

إسم المريض رباعياً : منى مصطفى محمد
المحافظة : القاهرة
الحى / المدينة : الشرايه
رقم البطاقة : 27608012702108 رقم التليفون : 01003801723
العمر(تاريخ الميلاد) : 01-08-1976 المهنة : ربه منزل
العنوان : 18ش عباس الزيات
تاريخ تسجيل الإستماره : 22-09-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6396] قسم العلاج : الاورام
التعليق على التشخيص : -----

GR1-E
روسى تاقىلىنى
لدى خىسەپ
مۇستافىي
صحة الامانيات المصرفية
اللجنة و مدير

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناقشة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

ملاحظات : ---

**رئيس
المجلس الطبي المتخصص**

--

Name: منى مصطفى محمد محمد National ID: 27608012702108	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدى و تحتاج الى Supportive and palliative care with follow-up
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
وزارة الداخلية

منى
مصطفى محمد محمد

١٨ شارع عباس الزيات
الشرابية - القاهرة

١٩٧٦/٠٨

٢٧٦.٠٨.٠١ ٢٧.٠٢١.٠٨

LB7724792

١- ٣٨٠/٧٢٣
١- ١٢٥/٦٩١



٢٠٢٥/٠٤ ٢٧٦.٠٨.٠١ ٢٧.٠٢١.٠٨

ربة منزل



أنثى مسلمة متزوجة
محمود مصطفى الصغير حسين احمد
البطاقة سارية حتى ٢٠٣٢/٠٤/٠٦

١



Name: Mona Moustafa Mohamed Mohamed
Referred by: Prof. Khaled Moustafa Al Kholi

ID: 5282281
Date: 09/08/2026

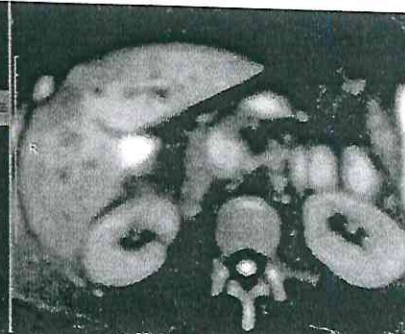
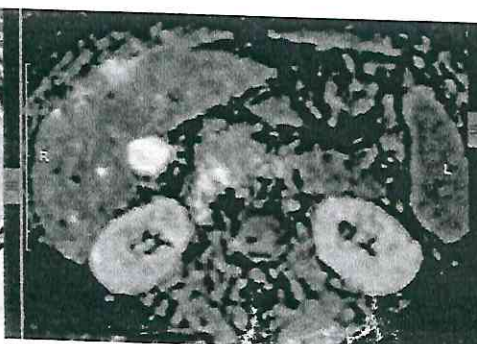
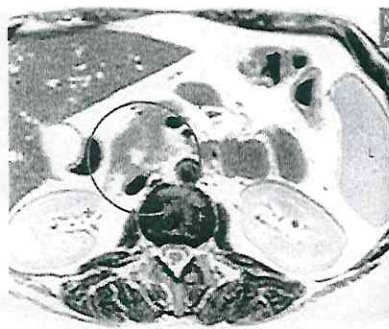
MR CHOLANGIOPANCRETOGRAPHY (MRCP)

MR TECHNIQUE:

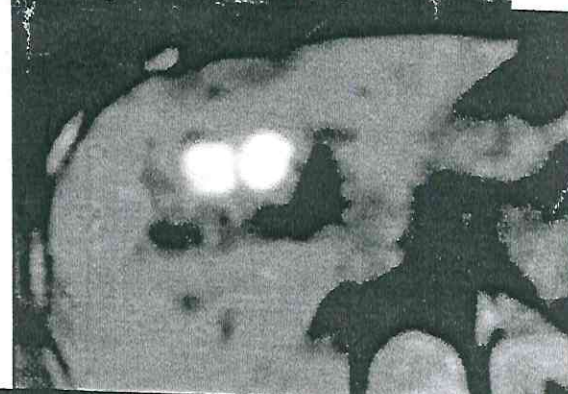
- Axial FSE/T2 and SPAIR images
- 2D and 3D heavy T2 MRCP images

(plates 1, 2).
(plates 3, 4).

MR FINDINGS:



- Moderate to marked dilatation of the intra and the extra hepatic biliary radicles with fusiform/lobulated appearance, associated sudden abrupt caliber change at the distal segment of the common bile duct about 2 cm from ampullar of vater. The CBD reached maximum axial diameter measuring 1.4 cm
- Poorly defined soft tissue lesion is seen epicentred upon the inferior aspect of pancreatic head/uncinate process showing diffusion restriction ; such lesion is seen partially inseparable from the 2nd part of the duodenum as well roughly measures about 1.8x1.4 cm
- No intra luminal filling defects seen within the dilated biliary channels.

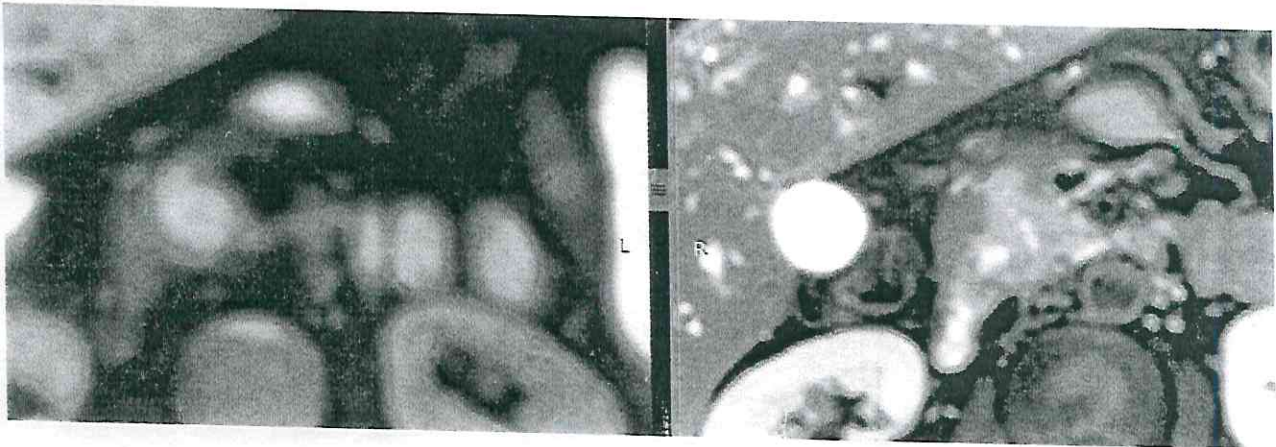


ID: 5282281

S C A N

- Two fairly defined hepatic focal lesions are noted within segment V and IVb measuring about 1.4 to 1.6 cm, they are seen eliciting low T1, high T2/SPAIR signal with peripheral diffusion restriction and surrounded by high parenchymal signal.
- The main pancreatic duct is mildly ectatic.
- Fluid level is noted within the distended gallbladder, likely attributed to sludge.

OPINION:



- Moderate to marked fusiform dilatation of the intra and extra hepatic biliary radicles as well as the CBD down to distal end that shows abrupt caliber tapering.
- Poorly defined soft tissue mass lesion seen epicentred upon inferior aspect of pancreatic head/uncinate process and partially inseparable from the duodenum showing diffusion restriction.
- Right hepatic lobe two well defined focal lesions; as described.

-Neoplastic process couldn't be ruled out for further dedicated assessment /further work up

Dr. Mohamed Ayman, MD

Dr. Sahar Saeed

جمهورية مصر العربية
بطاقة تحقيق الشخصية



منى
مصطفى محمد محمد

١٨ شارع عباس الزيات
الشرابية - القاهرة

١٩٧٦/٠٨

٢٧٦٠٨٠١٢٧٠٢١٠٨

LB7724792





S C A N

Name: Mona Moustafa Mohamed Mohamed
Referred by: Prof. Khaled Moustafa Al Kholy

ID: 5282281
Date: 09/08/2026

MR CHOLANGIOPANCRETOGRAPHY (MRCP)

MR TECHNIQUE:

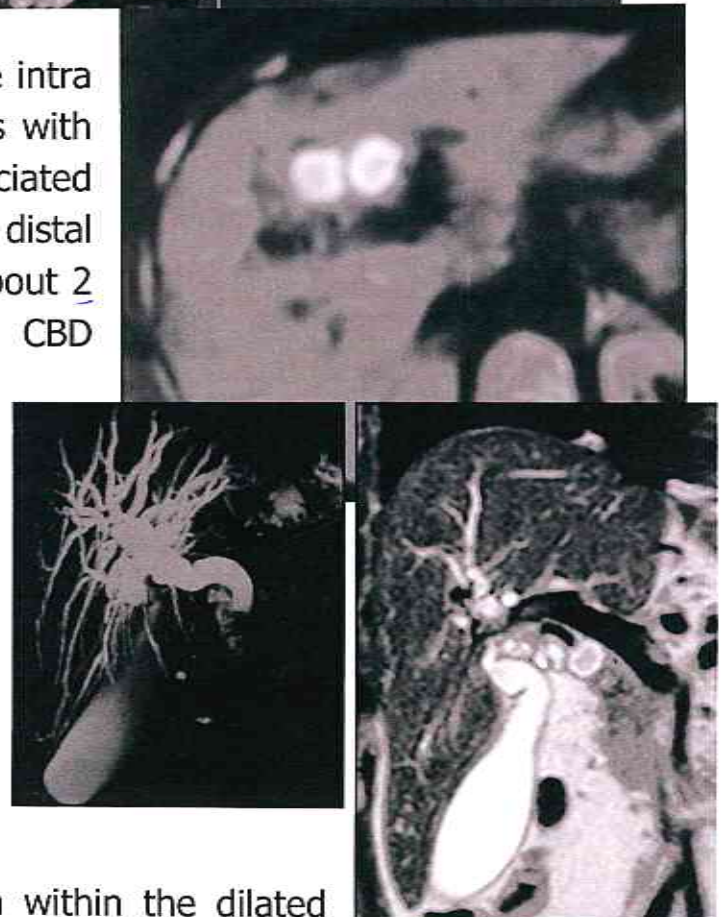
- Axial FSE/T2 and SPAIR images
- 2D and 3D heavy T2 MRCP images

(plates 1, 2).
(plates 3, 4).

MR FINDINGS:



- Moderate to marked dilatation of the intra and the extra hepatic biliary radicles with fusiform/lobulated appearance, associated sudden abrupt caliber change at the distal segment of the common bile duct about 2 cm from ampullar of vater. The CBD reached maximum axial diameter measuring 1.4 cm
- Poorly defined soft tissue lesion is seen epicentred upon the inferior aspect of pancreatic head/uncinate process showing diffusion restriction ; such lesion is seen partially inseparable from the 2nd part of the duodenum as well roughly measures about 1.8x1.4 cm
- No intra luminal filling defects seen within the dilated biliary channels.



ID: 5282281

AMG
ALFA MEDICAL GROUP

A MEMBER OF ALFA MEDICAL GROUP

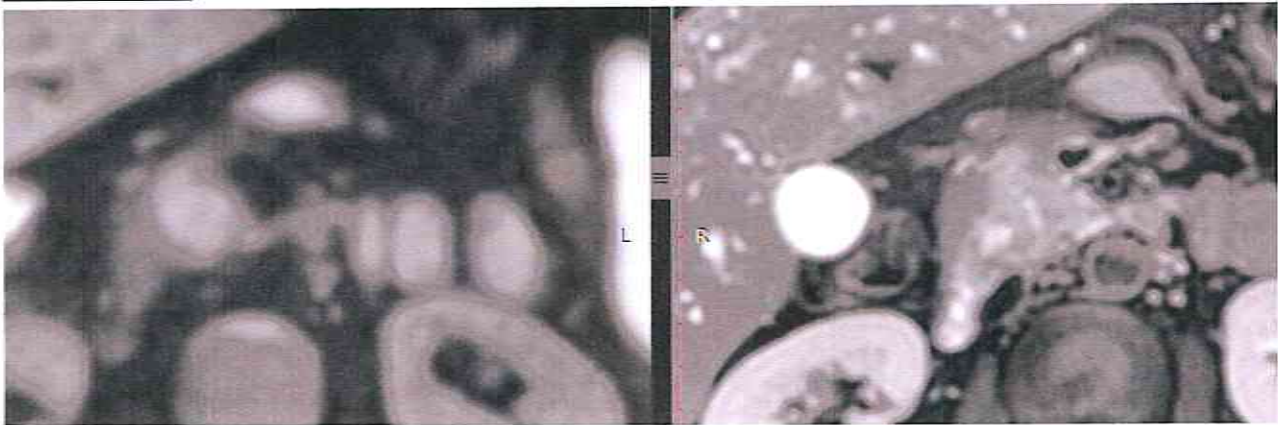
الفا سكان

16171

S C A N

- Two fairly defined hepatic focal lesions are noted within segment V and IVb measuring about 1.4 to 1.6 cm, they are seen eliciting low T1, high T2/SPAIR signal with peripheral diffusion restriction and surrounded by high parenchymal signal.
- The main pancreatic duct is mildly ectatic.
- Fluid level is noted within the distended gallbladder, likely attributed to sludge.

OPINION:



- **Moderate to marked fusiform dilatation of the intra and extra hepatic biliary radicles as well as the CBD down to distal end that shows abrupt caliber tapering.**
- **Poorly defined soft tissue mass lesion seen epicentred upon inferior aspect of pancreatic head/uncinate process and partially inseparable from the duodenum showing diffusion restriction.**
- **Right hepatic lobe two well defined focal lesions; as described.**

-Neoplastic process couldn't be ruled out for further dedicated assessment /further work up

Dr. Mohamed Ayman, MD

Dr. Sahar Saeed



الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومي للكبد والجهاز الهضمي
و الأمراض المعوية
وحدة منظار الكبد و الجهاز الهضمي

ERCP Report

Name: مني مصطفى محمد

ID: 276080127022108

Date: Monday 17, August 2026

Age: 50

Referring Doctor

Dear Prof.

Thank you for referring the case.

Indications

Malignant obstructive jaundice.

Medications

General anesthesia.

Instrument

Pentax EPK i7010.

Procedure Technique

In the prone position. After adequate sedation was achieved the patient was intubated under direct visualization and the scope advanced to the D2.

Endoscopic Findings

- Major Papilla seen in place.
- Cannulation of the papilla was possible by precut incision.

Radiological Findings

- Injection of contrast revealed markedly dilated CBD and IHBRs with distal CBD tight stricture.
- Insertion of 10 Fr, 10 cm stent with good biliary drainage.

Recommendation

- EUS.

Conclusion

- Malignant obstructive jaundice.
- Distal CBD stricture with dilated CBD and IHBRs.
- Insertion of 10 Fr, 10 cm stent.

Dr Bassem Amin, FASGE.

تقریر طبعی

$$\frac{|\lambda_{\text{eff}}|}{|\lambda_{\text{eff}}|}$$

تَحِيَّةٌ طَيِّبَةٌ وَبَعْدُ،

إيماء الطالب المقدم من
 بسان تقرير طبي للمريض:

بِسْمِ اللَّهِ الرَّحْمَنِ الرَّحِيمِ

تَنْسَرَفُ بَانَ نَوْضَحُ فِيمَا يَلِي تَقْرِيرًا عَنْ حَالَتِهِ :

تاريخ وساعة الدخول..... تاريخ وساعة الخروج.....

وبعد الكشف والفحوصات والمتابعة وجد انه:

بعد الكشف والفحوصات والمتابعة وجد انه:
The pt is known to have metastatic adenocarcinoma of unknown primary for routine labes & investigations before colonoscopy.
المرضى متابعين، وقد افادوا في تاريخياتهم وجود اكل
للحم، ففحصنا للتعبير الجيني، مظهر متغير،

تحريراً في

الوقت:

التاريخ:

21/9/2020

الطبيب المعالج

25/05/2022

۱۱، ۱۲

اسم المريض ثلاثي: هـ. هـ. هـ.
رقم الملف:

تقرير طبي

...../...../.....

تحية طبية وبعد،

إيماء لطلب المقدم من بشأن تقرير طبي للمريض:

بقسم رقم طبي

نتشرف بأن نوضح فيما يلي تقريراً عن حالته :

تاريخ وساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد أنه:
the pt is known to have infective aetiology.

المريض يعاني من حمى وصداع وتقيح ليل
معتدل، فلوليت للصبغ في سبيل العلاج.

تحريراً في

الوقت: التاريخ: / /

21/9/2026

الطبيب المعالج
د. أحمد محمد

يُعتقد،

Patient's Name: Mona Mustafa Mohammed Mohammed
Date: 2026-09-01
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

RADIOLOGY REPORT

Examination: CT of the chest, abdomen, and pelvis with intravenous contrast.

Comparison: MRCP dated 09/08/2026.

Findings:

Biliary System and Stent:

- A biliary stent is identified, likely migrated, with its distal portion located at the region of the third/fourth part of the duodenum and duodenojejunal junction.
- Persistent moderate to marked dilatation of the intrahepatic and extrahepatic biliary radicles is again demonstrated, with fusiform/lobulated appearance and abrupt caliber change at the distal common bile duct, similar to the prior MRCP.
- The common bile duct remains dilated, with maximum axial diameter measuring approximately 1.4 cm.
- No intraluminal filling defects are identified within the dilated biliary channels.

Pancreas and Duodenum:

- The previously described poorly defined soft tissue lesion at the inferior aspect of the pancreatic head/uncinate process is again suggested, remaining partially inseparable from the second part of the duodenum. Dedicated dynamic MRI is still recommended for further characterization.
- The pancreatic head appears prominent, measuring up to approximately 3 cm in thickness.
- The main pancreatic duct remains mildly ectatic.
- The superior mesenteric artery and vein are patent, with preserved fat plane between the mesenteric vessels and the suspected pancreatic head mass.

Liver:

- The liver is enlarged, measuring approximately 18 cm in craniocaudal span.
- Three well-defined hypodense hepatic focal lesions are now noted, slightly increased in number compared with the two lesions described on the prior MRCP. These lesions measure up to approximately 1.6 cm and may represent metastatic deposits or cholangitic abscesses; dedicated imaging is recommended for further assessment.
- No intrahepatic biliary pneumobilia is identified.

Spleen:

- Mild splenomegaly is noted, with the spleen measuring approximately 15 cm. No focal splenic lesion.

Chest:

Patient's Name:

Mona Mustafa Mohammed Mohammed

Date:

2026-09-01

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

- **Lungs:** No pulmonary nodules, masses, consolidation, or collapse.
- **Hila and mediastinum:** No lymphadenopathy or mass.
- **Pleura and pericardium:** No effusion or thickening.
- **Heart and great vessels:** Unremarkable.

Remaining Abdomen and Pelvis:

- **Kidneys:** Normal size, position, and parenchymal enhancement. No stones or hydronephrosis.
- **Adrenal glands:** Normal appearance.
- **Abdominal aorta and IVC:** Normal caliber, no aneurysm or dissection.
- **Lymph nodes:** No abdominal or pelvic lymphadenopathy.
- **Bowel:** No obstruction, wall thickening, or mass beyond the described duodenal region.
- **Urinary bladder and pelvic organs:** Unremarkable.
- **Peritoneal cavity:** No ascites or collections.
- **Bones:** No suspicious lytic or sclerotic lesions.

Impression:

1. Migrated biliary stent with its distal end at the region of the third/fourth part of the duodenum; persistent moderate to marked intra- and extrahepatic biliary dilatation, unchanged from the prior MRCP.
2. Suspected pancreatic head/uncinate process soft tissue lesion remains poorly defined; the preserved fat plane with the superior mesenteric vessels is a favorable sign. Dedicated dynamic contrast-enhanced MRI is strongly recommended for further assessment and to exclude an underlying neoplasm.
3. Three hepatic focal lesions, increased in number compared with the prior study; differential includes metastatic deposits and cholangitic abscesses. Dedicated liver imaging (MRI with hepatobiliary contrast or PET/CT) is recommended.
4. Hepatomegaly and mild splenomegaly.
5. No pulmonary or other distant metastatic disease identified on this CT.

Best regards,

Dr. Abdulaziz Oraby, MD.

Lecturer of interventional and radio-diagnosis.

جمهورية مصر العربية
بطاقة تحقيق الشخصية



منى
مصطفى محمد محمد

١٨ شارع عباس الزيات
الشرابية - القاهرة

١٩٧٦/٠٨

٢٧٦٠٨٠١٢٧٠٢١٠٨

LB7724792



Name: Mona Moustafa Mohamed Mohamed
Referred by: Prof. Khaled Moustafa Al Kholi

ID: 5282281
Date: 09/08/2026

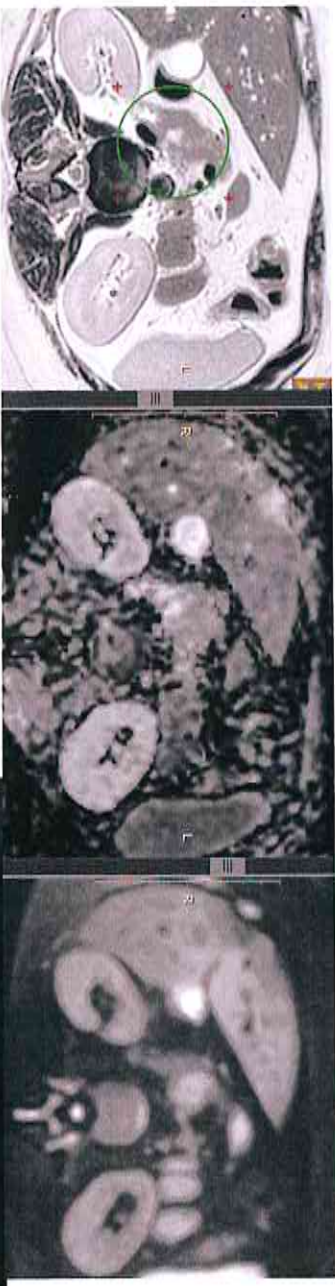
MR CHOLANGIOPANCRETOGRAPHY (MRCP)

MR TECHNIQUE:

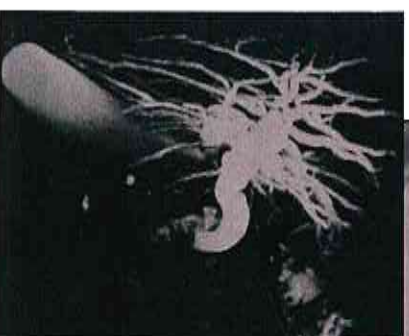
- Axial FSE/T2 and SPAIR images
- 2D and 3D heavy T2 MRCP images

(plates 1, 2).
(plates 3, 4).

MR FINDINGS:



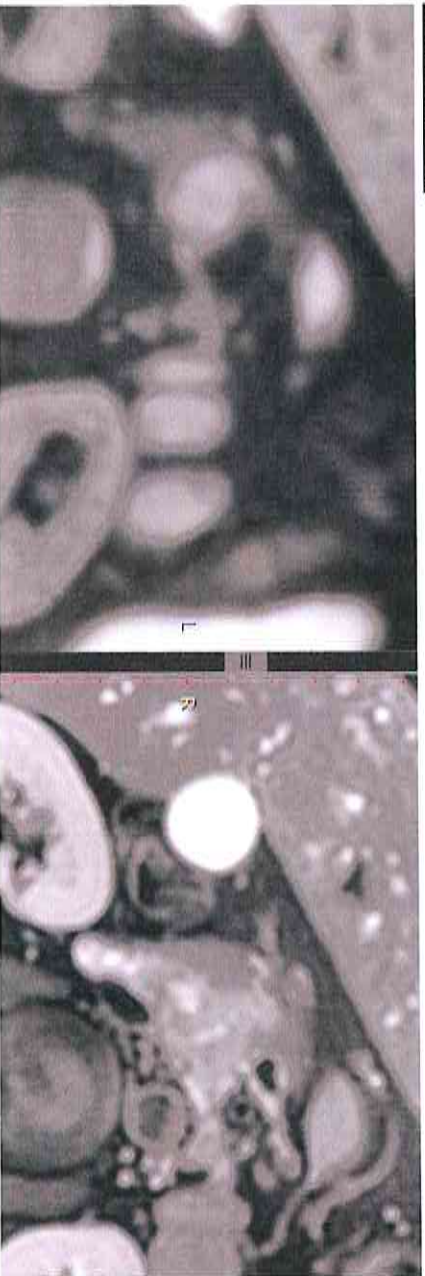
- Moderate to marked dilatation of the intra and the extra hepatic biliary radicles with fusiform/lobulated appearance, associated sudden abrupt caliber change at the distal segment of the common bile duct about 2 cm from ampullar of Vater. The CBD reached maximum axial diameter measuring 1.4 cm
- Poorly defined soft tissue lesion is seen epicentred upon the inferior aspect of pancreatic head/uncinate process showing diffusion restriction ; such lesion is seen partially inseparable from the 2nd part of the duodenum as well roughly measures about 1.8x1.4 cm
- No intra luminal filling defects seen within the dilated biliary channels.



S C A N

- Two fairly defined hepatic focal lesions are noted within segment V and IVb measuring about 1.4 to 1.6 cm, they are seen eliciting low T1, high T2/SPAIR signal with peripheral diffusion restriction and surrounded by high parenchymal signal.
- The main pancreatic duct is mildly ectatic.
- Fluid level is noted within the distended gallbladder, likely attributed to sludge.

OPINION:



- Moderate to marked fusiform dilatation of the intra and extra hepatic biliary radicles as well as the CBD down to distal end that shows abrupt caliber tapering.
 - Poorly defined soft tissue mass lesion seen epicentred upon inferior aspect of pancreatic head/uncinate process and partially inseparable from the duodenum showing diffusion restriction.
 - Right hepatic lobe two well defined focal lesions; as described.
- Neoplastic process couldn't be ruled out for further dedicated assessment /further work up*

Dr. Mohamed Ayman, MD

Dr. Sahar Saeed



NHTMRI



الهيئة العامة للمستشفيات و المعاملات الطبية
المعهد القومي للكبد و الجهاز الهضمي
و الأمراض المعدية
وحدة منظار الكبد و الجهاز الهضمي

ERCP Report

Name: مني مصطفى محمد

ID: 276080127022108

Date: Monday 17, August 2026

Age: 50

Referring Doctor

Dear Prof.

Thank you for referring the case.

Indications

Malignant obstructive jaundice.

Medications

General anesthesia.

Instrument

Entax EPK i7010.

Procedure Technique

In the prone position. After adequate sedation was achieved the patient was intubated under direct visualization and the scope advanced to the D2.

Endoscopic Findings

- Major Papilla seen in place.
- Cannulation of the papilla was possible by precut incision.

Radiological Findings

- Injection of contrast revealed markedly dilated CBD and IHBRs with distal CBD tight stricture.
- Insertion of 10 Fr, 10 cm stent with good biliary drainage.

Recommendation

- EUS.

Conclusion

- Malignant obstructive jaundice.
- Distal CBD stricture with dilated CBD and IHBRs.
- Insertion of 10 Fr, 10 cm stent.

Dr Bassem Amin, FASGE.